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Candazole Solution 1% w/v

NAME AND STRENGTH OF ACTIVE SUBSTANCE : Clotrimazole 1% w/v.

PRODUCT DESCRIPTION : A translucent colourless slight viscous liquid with a characteristic odour.

Other ingredient : Acrylates/Octylacrylamide Copolymer, Ethylcellulose, Sodium Hydroxide, Alcohol and Purified Water.

PHARMACODYNAMICS : Mechanism of Action: Clotrimazole acts against fungi by inhibiting ergosterol synthesis. Inhibition of ergosterol synthesis leads to structural and functional impairment of the fungal cytoplasmic membrane. Clotrimazole has a broad antimycotic spectrum of action in vitro and in vivo, which includes dermatophytes, yeasts, moulds, etc. Under appropriate test conditions, the MIC values for these types of fungi are in the region of less than 0.062-8.0 µg/ml substrate. The mode of action of clotrimazole is primarily fungistatic or fungicidal depending on the concentration of clotrimazole at the site of infection. In vitro activity is limited to proliferating fungal elements; fungal spores are only slightly sensitive. In addition to its antimycotic action, clotrimazole also acts on gram-positive microorganisms (Streptococci / Staphylococci / Gardnerella vaginalis), and gram-negative microorganisms (Bacteroides). In vitro clotrimazole inhibits the multiplication of Corynebacteria and gram-positive cocci - with the exception of Enterococci - in concentrations of 0.5-10µg/ml substrate. Primarily resistant variants of sensitive fungal species are very rare; the development of secondary resistance by sensitive fungi has so far only been observed in very isolated cases under therapeutic conditions.

PHARMACOKINETICS : Pharmacokinetic investigations after dermal application have shown that clotrimazole is minimally absorbed from the intact or inflamed skin into the human blood circulation. The resulting peak serum concentrations of clotrimazole were below the detection limit of 0.001 mcg/ml, suggesting that clotrimazole applied topically is unlikely to lead to measurable systemic effects or side effects.

INDICATIONS : It is used to treat all fungal skin infections due to dermatophytes, yeasts, moulds and other fungi.

CONTRAINDICATIONS : Hypersensitivity to clotrimazole, or any component in the preparation.

ADVERSE EFFECTS : As the listed undesirable effects are based on spontaneous reports, assigning an accurate frequency of occurrence for each is not possible. Immune system disorders: allergic reaction (syncope, hypotension, dyspnea, urticaria). Skin and subcutaneous tissue disorders: blisters, discomfort/pain, oedema, erythema, irritation, peeling/exfoliation, pruritus, rash, stinging/burning.

DRUG INTERACTION : Unknown.

WARNINGS AND PRECAUTIONS : Avoid contact with eyes and mucous membranes (for external use only). Not for ophthalmic use.

Fertility: No human studies of the effects of clotrimazole on fertility have been performed; however, animal studies have not demonstrated any effects of the drug on fertility.

Effects on ability to drive and use machines: Clotrimazole has no or negligible influence on the ability to drive or use of machines.

Preclinical safety data: Non-clinical data reveal no special hazard for humans based on studies of repeated dose toxicity, genotoxicity and carcinogenicity. Clotrimazole was not teratogenic in

reproductive toxicity studies in mice, rats and rabbits. In rats high oral doses were associated with maternal toxicity, embryotoxicity, reduced fetal weights and decreased pup survival. In rats clotrimazole and/or its metabolites were secreted into milk at levels higher than in plasma by a factor of 10 to 20 at 4 hrs after administration, followed by a decline to a factor of 0.4 by 24 hrs.

PREGNANCY AND LACTATION :

Pregnancy: There is a limited amount of data from the use of clotrimazole in pregnant women. Animal studies with clotrimazole have shown reproductive toxicity at high oral doses. At the low systemic exposures of clotrimazole following topical treatment, harmful effects with respect to reproductive toxicity are not predicted. Clotrimazole can be used during pregnancy, but only under the supervision of a physician or midwife.

Lactation: It is not known whether clotrimazole is excreted in breast milk after topical administration. Available pharmacodynamic/toxicological data in animals have shown excretion of clotrimazole/metabolites in milk after intravenous administration. A risk to the suckling child cannot be excluded. A decision must be made whether to discontinue breast-feeding or to discontinue/abstain from clotrimazole therapy taking into account the benefit of breast-feeding for the child and the benefit of therapy for the woman.

ROUTE OF ADMINISTRATION : Topical.

RECOMMENDED DOSAGE : Wash and dry affected area. The solution should be thinly and evenly applied to the affected area 2 or 3 times a day and gently rubbed in. A few drops are enough to treat an area of about the size of the hand. To prevent relapse, treatment should be continued for at least 2 weeks after all clinical signs have disappeared. Attention to hygiene is important in the management of fungal diseases, particularly of the folds and the feet. There is no separate dosage schedule for the elderly or the young.

SYMPTOMS AND TREATMENT FOR OVERDOSE AND ANTIDOTE(S) : No risk of acute intoxication is seen as it is unlikely to occur following a single dermal application of an overdose (application over a large area under conditions favourable to absorption) or inadvertent oral ingestion. There is no specific antidote. However, in the event of accidental oral ingestion, routine measures such as gastric lavage should be performed only if clinical symptoms of overdose become apparent (e.g. dizziness, nausea or vomiting). Gastric lavage should be carried out only if the airway can be protected adequately.

PACKING : 10ml glass bottle.

STORAGE : Keep container well closed. Protect from light. Store below 30°C. Keep out of reach of children. Recommended shelf-life: 3 years.

MANUFACTURED BY :

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